A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

17 April 2026

Lynette Waring

Dear Lynette Waring,

Re: Ms Sarah Waring
DOB: 1993-12-01
Address: 2/28 Belgrave Street, HAWTHORN, VIC, 3122

Summary

- **Diagnosis of eosinophilic oesophagitis confirmed on histology.**
- **Significant oesophageal remodelling and fibrostenotic changes noted.**
- **Commencement of dual therapy with high-dose proton pump inhibitor (PPI) and orodispersible budesonide (Jorveza).**
- **Plan for six-month medication supply to facilitate extended overseas travel.**
- **Repeat gastroscopy required in January 2025 to evaluate histological remission for ongoing PBS authority.**
- **No action required by you.**

I had the pleasure of reviewing Sarah following her recent gastroscopies. As you are aware, she has undergone three procedures which have now definitively confirmed a diagnosis of eosinophilic oesophagitis (EoE). Both the endoscopic appearance and the histological findings are consistent with this diagnosis. Of note, there is evidence of significant remodelling and scar tissue within the oesophagus, which accounts for her symptoms of dysphagia and food bolus obstruction.

We had a lengthy discussion regarding management options, particularly in the context of Sarah's upcoming travel to South America in late June. While dietary interventions, such as the six-food elimination diet, were discussed, we agreed this is impractical given her travel plans. Furthermore, while high-dose PPI therapy alone can induce remission in approximately 40-60% of cases, given the degree of fibrostenotic change, I believe a more aggressive pharmacological approach is warranted to achieve rapid symptomatic and histological control.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan The plan is to commence Sarah on combination therapy. This will include a high-dose PPI twice daily, taken before breakfast and dinner. In addition, I have prescribed Jorveza (orodispersible budesonide) 1mg twice daily. I spent time instructing Sarah on the correct administration of Jorveza, emphasizing that the tablet should be placed on the tongue and pressed against the roof of the mouth to dissolve, with the resulting slurry swallowed slowly. She is to remain nil by mouth for 30 minutes following each dose to ensure adequate mucosal contact. I have also counselled her on the 10% risk of developing oral or oesophageal candidiasis. Logistically, I will apply for a PBS authority script closer to her departure date on the 29th of June to ensure she has a sufficient six-month supply for her travels. I have advised her to remain cautious with her eating habits while abroad and discussed the use of carbonated beverages as a first-aid measure should she experience a food bolus sensation. Sarah will contact the clinic upon her return in January 2027. At that time, we will arrange a surveillance gastroscopy. This is clinically indicated to assess treatment response and is a regulatory requirement to secure ongoing government subsidisation for Jorveza. I will review her in the clinic once those results are available.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Amanda Richards, Dr Pankajam Kandasamy

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